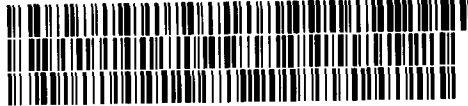


Procedure Date:
Procedure Physician:
Attending Physician/Copies To:

1CD-0-3
Carcinoma, medullary, NOS 8510/3
Site: Breast, NOS C50.9 1/21/11

UUID:7B1B31D2-0CC5-4EE6-8AF3-16DF726BE13F
TCGA-BH-A1FC-01A-PR

Redacted



PATIENT HISTORY:

* DATE OF LMP: *

DATE OF LAST DELIVERY: *

PRE-OP DIAGNOSIS: R BREAST CANCER

POST-OP DIAGNOSIS: SAME

OPERATIVE PROCEDURE: R SEGM MASTECTOMY; R AXILLARY DISSECTION

CLINICAL HISTORY: *

MATERIAL SUBMITTED: A) RIGHT BREAST BIOPSY/MASS/SEGMENTAL MASTECTOMY, PROCUREMENT BY SURGICAL PROCEDURE

B) RIGHT AXILLA, PROCUREMENT BY SURGICAL PROCEDURE

INTRAOPERATIVE CONSULTATION:

CONSULT: Right breast tissue; 9.5 by 8.5 by 1.5 cm. Tumor; 1.3 by 1.2 by 1.0 cm. Margins are grossly free of tumor.

FINAL DIAGNOSIS:

PREVIOUS REPORT: [REDACTED]

A) RIGHT BREAST (SEGMENTAL MASTECTOMY):

- INFILTRATING MEDULLARY CARCINOMA (A1, A2), 1.3 BY 1.2 BY 1.0 CM, POOR HISTOLOGIC AND NUCLEAR GRADE (SEE NOTE)

- MARGINS OF RESECTION ARE FREE OF TUMOR

- PROLIFERATIVE FIBROCYSTIC CHANGES

B) ONE OF TWENTY-THREE (1/23) RIGHT AXILLARY LYMPH NODES WITH METASTATIC CARCINOMA AND EXTRACAPSULAR EXTENSION (SEE NOTE)

NOTE: The tumor is high grade with syncytial pattern and extensive lymphocytic reaction. No in situ carcinoma is present. One large lymph node is completely replaced by tumor with extracapsular extension (B7).

ER/PR immunoperoxidase assay and Her-2/Neu testing will be performed on block A2.

SUPPLEMENTAL REPORT

(ER/PR'S)

My signature below is attestation that I have reviewed all slides and agree with the findings as noted below.

IMMUNOPEROXIDASE IDENTIFICATION OF ESTROGEN AND PROGESTERONE RECEPTORS IS CARRIED OUT ON SLIDE "A2". NO DISTINCT INTRANUCLEAR STAINING IS IDENTIFIED FOR EITHER RECEPTOR. THEREFORE, BOTH ARE INTERPRETED AS NEGATIVE.

Pathologist

SUPPLEMENTAL REPORT

(HER-2/NEU)

My signature below is attestation that I have reviewed all slides and agree with the findings as noted below.

AS PER THE REQUEST OF [REDACTED] OF [REDACTED] CARRIED OUT ON PREVIOUS MAGEE SURGICAL

DAKO'S POLYCLONAL ANTIBODY A485 (DIRECTED AGAINST THE INTRACELLULAR DOMAIN OF c-erbB2) WITHOUT ANTIGEN RETRIEVAL. NO DISTINCT COMPLETE MEMBRANE STAINING IS IDENTIFIED. THEREFORE, c-erbB2 (HER-2/NEU) IS INTERPRETED AS NEGATIVE.

Criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor Site Discrepancy		X
HIFAA Discrepancy		X
Prior Malignancy History		
Dual/Synchronous Primary Nodes		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials: [Signature]	Date Reviewed: 1/21/11	